

Access guide — pancreatic-recurrent-kras-g12r-m8f3

How to access each therapy: trial contacts + manufacturer medical-info

Generated 2026-08-21

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How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a [Verified](#) date; re-screen before relying on a specific contact or trial slot.

Summary

#	Intervention	Modality	Access status	Regulatory	Recommended first action
1	olaparib (Lynparza)	small_molecule	Off-label use	FDA-approved (December 2019) for maintenance treatment of germline BRCA1/2-mutated metastatic PDAC after at least 16 weeks of first-line platinum-based chemotherapy without progression (POLO indication). The patient becomes on-label (standard_of_care) once a germline BRCA1/2 result is positive and a platinum induction with ≥ 16 weeks of disease control is documented; until both gates clear, this is off-label / pre-gate. Also FDA-approved across BRCA-mutated ovarian, HER2-negative breast, and HRR-mutated mCRPC indications. EMA-approved (2020) for the same PDAC maintenance indication.	Order a germline panel covering BRCA1, BRCA2, PALB2, and the broader HBOC / Lynch / Li-Fraumeni axes. The result is the load-bearing biomarker for olaparib coverage under the POLO indication.

2	palbociclib (Ibrance) + MEK/ERK or IGF1R combination	small_molecule	Off-label use + Clinical trial	Palbociclib is FDA-approved (2015) and EMA-approved (2016) for HR-positive HER2-negative advanced breast cancer in combination with endocrine therapy. Not approved for PDAC. The MEK/ERK or IGF1R combination partners would also be used off-label.	Track ADOPT (NCT06813079) recruitment status through the listed central contact (Robert C. Grant, MD, robert.grant@uhn.ca, 416-946-4501 ext. 3308). The PDO-guided route is the most defensible structural fit.
3	rucaparib (Rubraca)	small_molecule	Off-label use	FDA-approved for maintenance treatment of recurrent epithelial ovarian / fallopian tube / primary peritoneal cancer in response to platinum-based chemotherapy (2018) and for adults with deleterious germline / somatic BRCA-mutated metastatic castration-resistant prostate cancer previously treated with androgen receptor-directed therapy (regular approval December 2025). EMA-approved in ovarian cancer indications. Not FDA-approved for PDAC.	Order the same germline / somatic HRR panel ordered for olaparib eligibility. A positive BRCA1/2 or PALB2 hit on either germline or somatic testing opens this lane via the rucaparib precedent.
4	ELI-002 7P amphiphile mKRAS peptide vaccine	vaccine	Clinical trial + Not yet accessible	Investigational. Phase 1/2 program with positive AMPLIFY-201 readout (Wainberg Nat Med 2025); no FDA, EMA, or PMDA approval.	This pathway is not enrollable in the patient's current clinical state. Listed for completeness.

5	PF-07934040 (Pfizer pan-KRAS inhibitor)	small_molecule	Clinical trial	Investigational. Phase 1 first-in-human; no regulatory approval. IND-cleared, no Breakthrough or Fast Track designation on the public record.	Call the Pfizer CT.gov call center (1-800-718-1021) or email with the patient's KRAS G12R status to ask which PDAC site has open slots in Part 2a Cohort A1 (2L+ monotherapy) and Part 2b Cohort A2 (1L gem/nab).
6	anvumetostat (AMG 193) + daraxonrasib	small_molecule	Clinical trial	Investigational. Both agents are pre-approval. Anvumetostat is in phase 1b; daraxonrasib carries FDA Breakthrough Therapy designation in PDAC but no approval.	Order MTAP IHC (or NGS copy-number) on the archival tumor block. CDKN2A and MTAP co-delete in roughly 80-90% of homozygous CDKN2A losses because the two genes sit adjacent on 9p21, so this is a high-yield test.
7	avutometinib (VS-6766) + defactinib (FAK inhibitor) +	small_molecule	Clinical trial + Not yet accessible	Avutometinib + defactinib carries FDA accelerated approval (May 2025) for adult patients with recurrent KRAS-mutated low-grade serous ovarian cancer who have received prior systemic therapy, marketed as Avmapki Fakzynja Co-pack. No FDA or EMA approval in PDAC. Gem/nab is FDA-approved standard-of-care backbone.	Email clinicaltrials@verastem.com or call 1-877-VSTM-ONC (1-877-878-6662) to ask whether RAMP-205 has reopened or whether a successor PDAC study is planned.
8	daraxonrasib (RMC-6236)	small_molecule	Clinical trial	Investigational. FDA Breakthrough Therapy designation for previously treated metastatic KRAS G12X PDAC (2024). No FDA, EMA, or PMDA approval yet; RASolute-302 phase 3 (NCT06625320) is the registrational trial and is active not recruiting.	Email medinfo@revmed.com or call 1-844-2-REVMED to ask which RMC-6236-001 (NCT05379985) or RMC-GI-102 (NCT06445062) site is closest and currently accepting screening slots.

9	daraxonrasib (RMC-6236) + mFOLFIRINOX or	small_molecule	Clinical trial	Investigational combination. Daraxonrasib carries FDA Breakthrough Therapy designation in PDAC but is not approved. The chemotherapy backbones (mFOLFIRINOX, gem/nab) are FDA-approved standard-of-care components used per protocol.	Call Revolution Medicines medical information (1-844-2-REVMED) or email medinfo@revmed.com asking specifically about the RMC-GI-102 PDAC chemo-combination subprotocol and the nearest open site.
10	zoldonrasib (RMC-7977) + ivonescimab	small_molecule	Clinical trial	Investigational. Zoldonrasib is pre-approval; ivonescimab (Akeso AK112) is approved in China for first-line PD-L1-positive NSCLC and EGFR-mutant NSCLC post-TKI but has no FDA or EMA approval. The combination has no regulatory filing.	Email medinfo@revmed.com asking whether NCT07397338 has an open PDAC dose-expansion cohort and at which sites.

Off-label use (3)

1. olaparib (Lynparza)

Aliases: olaparib, AZD2281, Lynparza

Modality: small_molecule · **Regulatory:** FDA-approved (December 2019) for maintenance treatment of germline BRCA1/2-mutated metastatic PDAC after at least 16 weeks of first-line platinum-based chemotherapy without progression (POLO indication). The patient becomes on-label (standard_of_care) once a germline BRCA1/2 result is positive and a platinum induction with ≥ 16 weeks of disease control is documented; until both gates clear, this is off-label / pre-gate. Also FDA-approved across BRCA-mutated ovarian, HER2-negative breast, and HRR-mutated mCRPC indications. EMA-approved (2020) for the same PDAC maintenance indication. ·

Guidelines: NCCN Pancreatic Adenocarcinoma v2.2026 category 1 for germline BRCA1/2-mutated metastatic PDAC maintenance after at least 16 weeks of platinum without progression. The only NCCN category 1 listing in this dossier. · **Geographic scope:** United States, EU, plus most ICH regions. Indication-matched access requires the patient to fall within the POLO label. · **Verified:** 2026-05-12

Commercial prescription rather than trial. Olaparib is FDA / EMA approved on-label for germline BRCA1/2 metastatic PDAC maintenance, but the patient has two gates to clear first: a germline BRCA1/2 (and ideally PALB2) panel result, and at least 16 weeks of platinum disease control on a future platinum-induction regimen.

Both gates are outside Libby's scope; until they resolve this is an indication-matched-when-gates-open option rather than an immediate one.

Next steps

1. Order a germline panel covering BRCA1, BRCA2, PALB2, and the broader HBOC / Lynch / Li-Fraumeni axes. The result is the load-bearing biomarker for olaparib coverage under the POLO indication.
2. If germline BRCA1/2 or PALB2 returns positive, the treating team will need to consider a platinum-based induction (mFOLFIRINOX or NALIRIFOX) before olaparib maintenance becomes eligible under the FDA label.
3. Contact AstraZeneca Access 360 (1-844-275-2360) early for prior-authorization support and the MyLYNPARZA nurse line (1-800-770-8337) once therapy starts.
4. If the germline panel returns negative on BRCA1/2 but positive on PALB2, discuss with the treating team whether the rucaparib precedent (Reiss 2021) supports a non-label PARP route; olaparib coverage remains label-restricted to BRCA1/2.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02134195		completed	no	—
NCT04828334		recruiting	no	—

Manufacturer / sponsor contact

- **Company:** AstraZeneca (in collaboration with Merck & Co.)
- **Med info phone:** 1-800-236-9933
- **Product info:** <https://medicalinformation.astrazeneca-us.com/home/prescribing-information/lynparza-tablets.html>
- **Compassionate / expanded access:** <https://www.astrazenecaclinicaltrials.com/our-clinical-trials/expanded-access/>
- **Notes:** AstraZeneca US Medical Information handles unsolicited product-information queries at 1-800-236-9933. Patient-access support routes through AstraZeneca Access 360 (1-844-275-2360) for prior-authorization help and through the MyLYNPARZA nurse line (1-800-770-8337) for ongoing therapy support.

Payer / coverage: Indication-matched coverage is contingent on a documented germline BRCA1/2 result plus a documented platinum induction with at least 16 weeks of disease control. Medicare Part D and most commercial payers cover the POLO indication under the FDA label; AstraZeneca Access 360 handles prior-authorization and copay-assistance support. Without a germline BRCA result, payer coverage drops to off-label / case-by-case review.

Notes: POLO showed PFS HR 0.53 (mPFS 7.4 vs 3.8 mo); interim OS HR 0.91 was not statistically significant, so the payoff under the label is durable disease control rather than confirmed OS improvement. PARP-class MDS / AML risk runs roughly 1.5% on prolonged exposure.

2. palbociclib (Ibrance) + MEK/ERK or IGF1R combination

Aliases: palbociclib, PD-0332991, Ibrance

Modality: small_molecule · **Regulatory:** Palbociclib is FDA-approved (2015) and EMA-approved (2016) for HR-positive HER2-negative advanced breast cancer in combination with endocrine therapy. Not approved for PDAC. The MEK/ERK or IGF1R combination partners would also be used off-label. · **Guidelines:** Not listed in NCCN Pancreatic Adenocarcinoma v2.2026 for the CDKN2A-loss or CCND3 indication. The NCI-MATCH Z1C basket (CDK4/6-amplified solid tumors, O'Hara 2025 PMID 39437014) returned ORR 4%, which actively argues against single-agent CDK4/6i in this axis. · **Geographic scope:** United States and EU; palbociclib is commercially available wherever it has an HR-positive breast cancer indication. · **Verified:** 2026-05-12

Off-label prescription is technically available because palbociclib is FDA-approved (for HR-positive breast cancer); ADOPT (NCT06813079) is the PDO-guided trial route but is not yet recruiting. Neither path is well-supported: NCI-MATCH Z1C returned ORR 4% in CDK4/6-amplified solid tumors, and the combination rationale rests on preclinical organoid data (Knudsen 2023) without PDAC clinical validation.

Next steps

1. Track ADOPT (NCT06813079) recruitment status through the listed central contact (Robert C. Grant, MD, robert.grant@uhn.ca, 416-946-4501 ext. 3308). The PDO-guided route is the most defensible structural fit.
2. Discuss with the treating team whether MTAP testing redirects toward the PRMT5 axis (MTAPESTRY-103, NCT06360354) before pursuing CDK4/6 inhibition off-label.
3. If off-label palbociclib is still pursued (typically as part of a combination with a MEKi or ERKi), the institutional pharmacy and payer prior-authorization process is the load-bearing step. Pfizer Medical Information (1-800-438-1985) handles product-information queries.
4. Treat this lane as the lowest-priority option in the dossier given the negative Z1C precedent; rank-1 through rank-4 lanes are operationally and biologically stronger paths.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06823079		not yet recruiting	unconfirmed	Robert C. Grant, MD; robert.grant@uhn.ca; 416-946-4501 ext. 3308
NCT03227606		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Pfizer
- **Med info phone:** 1-800-438-1985
- **Product info:** <https://www.pfizermedical.com/>
- **Compassionate / expanded access:** <https://www.pfizer.com/science/clinical-trials/expanded-access>
- **Notes:** Palbociclib is commercially available for the HR-positive HER2-negative breast cancer indication and can be obtained for off-label use through standard specialty-distribution channels, subject to institutional pharmacy and payer review. Pfizer Medical Information at 1-800-438-1985 handles product-information queries; expanded-access for an approved drug is handled as off-label prescription rather than EAP.

Payer / coverage: Off-label use for PDAC carries heavy payer review and is generally not reimbursed without prior authorization plus an appeal letter citing the CDKN2A-loss / CCND3 alteration rationale. The Z1C basket negative readout (O'Hara 2025, ORR 4%) weakens the payer-justification case for single-agent use; combination logic (palbociclib + MEKi or ERKi or IGF1Ri) is preclinical-only and unsupported by published clinical data in

PDAC. Manufacturer patient-assistance through Pfizer Oncology Together may help with copay support if coverage is obtained.

Notes: Useful as informational context for the CDKN2A / CCND3 axis rather than a recommended action. The mechanism-fit (CDKN2A loss derepresses CDK4/6; CCND3 amplification drives CDK4/6 dependency) is textbook, but the basket precedent is negative and the combination logic has not been translated to PDAC clinically.

3. rucaparib (Rubraca)

Aliases: rucaparib, Rubraca

Modality: small_molecule · **Regulatory:** FDA-approved for maintenance treatment of recurrent epithelial ovarian / fallopian tube / primary peritoneal cancer in response to platinum-based chemotherapy (2018) and for adults with deleterious germline / somatic BRCA-mutated metastatic castration-resistant prostate cancer previously treated with androgen receptor-directed therapy (regular approval December 2025). EMA-approved in ovarian cancer indications. Not FDA-approved for PDAC. · **Guidelines:** Not yet NCCN category 1 for PDAC. The somatic-BRCA / PALB2 PDAC maintenance use rests on the Reiss 2021 phase 2 (single-arm n=46) and is not codified at the same level as olaparib in germline-BRCA disease. · **Geographic scope:** United States and EU; off-label availability tracks the approved-indication distribution. · **Verified:** 2026-05-12

Off-label PDAC use only. Rucaparib is FDA-approved for BRCA-mutated ovarian and prostate cancers but not for PDAC; the Reiss 2021 phase 2 (n=46, mPFS 13.1 mo, mOS 23.5 mo) is the precedent. The same two gates as olaparib apply (germline / somatic BRCA or PALB2 result plus a platinum induction with disease response), and on top of that this is an off-label prescription rather than a label-indicated one.

Next steps

1. Order the same germline / somatic HRR panel ordered for olaparib eligibility. A positive BRCA1/2 or PALB2 hit on either germline or somatic testing opens this lane via the rucaparib precedent.
2. If germline BRCA1/2 is positive, default to olaparib under the POLO indication rather than rucaparib off-label; rucaparib becomes the relevant option when the hit is somatic-only or on PALB2.
3. Contact pharma& medical information at medinfo.us@pharmaand.com with the patient's biomarker result and prior-platinum status to ask about prior-authorization support and any patient-assistance program.
4. Discuss with the treating team and institutional pharmacy before submitting the prior-authorization request; a biomarker-justified appeal letter citing Reiss 2021 (JCO PMID 33970687) is typically required.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03120670		completed	no	—

Manufacturer / sponsor contact

- **Company:** pharma& (pharmaand GmbH)
- **Med info email:** medinfo.us@pharmaand.com
- **Product info:** <https://www.rubraca.com/>

- **Notes:** pharma& acquired the global rucaparib rights from Clovis Oncology after Clovis's 2022 bankruptcy. US medical-information inquiries route through medinfo.us@pharmaand.com; adverse-event reports route through pv@pharmaand.com. Headquarters phone (Vienna) is +43/1/3560006.

Payer / coverage: PDAC use is off-label. Payer coverage for off-label PARP use in BRCA / PALB2 PDAC is case-by-case and typically requires a biomarker-justified appeal letter citing Reiss 2021 (JCO PMID 33970687) and prior platinum response. Coverage is generally easier to obtain than for unselected off-label use because the mechanistic rationale is well-established and an FDA-approved competitor exists for the germline BRCA subset.

Notes: Reiss 2021 was a single-arm phase 2 with PFS-rate at 6 months as the registered primary endpoint; the mPFS and mOS figures are secondary readouts and do not carry RCT weight. Same PARP-class MDS / AML risk applies on prolonged exposure.

Clinical trial (7)

4. ELI-002 7P amphiphile mKRAS peptide vaccine

Aliases: ELI-002 7P, ELI-002, AMPLIFY-7P

Modality: vaccine · **Regulatory:** Investigational. Phase 1/2 program with positive AMPLIFY-201 readout (Wainberg Nat Med 2025); no FDA, EMA, or PMDA approval. · **Guidelines:** Not listed in NCCN or ESMO PDAC guidelines. KRAS vaccines have no guideline category yet. · **Geographic scope:** United States, plus select EU sites under AMPLIFY-7P. Trial enrollment is closed. · **Verified:** 2026-05-12

Not currently accessible for this patient. The peptide panel covers G12R cleanly and AMPLIFY-201 read positively (CD4 / CD8 KRAS-specific T-cell responses in 71% of evaluable patients), but AMPLIFY-7P enrolls only MRD-positive resected PDAC and the patient has overt recurrence. The trial is also active not recruiting. Logged so the file shows the option was considered and ruled out on indication, not mechanism.

Next steps

1. This pathway is not enrollable in the patient's current clinical state. Listed for completeness.
2. If a curative-intent salvage resection becomes feasible later, revisit ELI-002 maintenance via a follow-on study. Elicio may open an overt-disease cohort; track at info@elicio.com.
3. If pursued as a last-resort path, the treating physician would submit an expanded-access request under Elicio's posted policy (<https://elicio.com/expanded-access-policy/>); approval outside the resected MRD window is unlikely without a clinical-state-matched precedent.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05716264		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Elicio Therapeutics

- **Med info phone:**+1 857 209-0050
- **Med info email:**info@elicio.com
- **Product info:**<https://www.elicio.com/>
- **Compassionate / expanded access:**<https://elicio.com/expanded-access-policy/>
- **Notes:** Elicio publishes an expanded-access policy at the URL above. Small biotech; the general line handles both medical and corporate inquiries. Compassionate use for an overt-recurrent patient outside the AMPLIFY-7P window would require direct medical-affairs review and is unlikely to be granted without a clinical-state-matched precedent.

Payer / coverage: Investigational. No payer coverage outside trial enrollment.

Notes: Cleanest safety profile in the dossier (no DLTs, no grade-3+ vaccine-attributed AEs across AMPLIFY-201, n=25). Mechanism is unusually clean for G12R; the indication mismatch is the binding constraint, not biology.

5. PF-07934040 (Pfizer pan-KRAS inhibitor)

Aliases: PF-07934040, PF07934040

Modality: small_molecule · **Regulatory:** Investigational. Phase 1 first-in-human; no regulatory approval. IND-cleared, no Breakthrough or Fast Track designation on the public record. · **Guidelines:** Not listed in NCCN Pancreatic Adenocarcinoma v2.2026. Trial-enrollment principle is the guideline-fit framing. · **Geographic scope:** United States, with select EU and Asia-Pacific sites. The Pfizer call-center routes by geography. · **Verified:** 2026-05-12

Trial-only. NCT06447662 is recruiting in PDAC and explicitly lists G12R in eligibility. Operationally it is a phase 1 dose-finding slot, so safety data are still being mapped; the dossier ranks this as the backup pan-KRAS lane if a daraxonrasib slot is unavailable.

Next steps

1. Call the Pfizer CT.gov call center (1-800-718-1021) or email ClinicalTrials.gov_Inquiries@pfizer.com with the patient's KRAS G12R status to ask which PDAC site has open slots in Part 2a Cohort A1 (2L+ monotherapy) and Part 2b Cohort A2 (1L gem/nab).
2. Confirm residual sensory neuropathy from prior FOLFIRI is below grade 2 before screening; irinotecan-based regimens are usually neuropathy-sparing but the exclusion is hard.
3. Clarify with the site coordinator whether adjuvant FOLFIRI counts as prior systemic therapy for the 2L+ cohort, or whether it leaves the patient eligible for the 1L gem/nab arm.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06447662		recruiting	likely	Pfizer CT.gov Call Center; 1-800-718-1021

Manufacturer / sponsor contact

- **Company:** Pfizer
- **Med info phone:** 1-800-438-1985
- **Product info:** <https://www.pfizermedical.com/>
- **Compassionate / expanded access:** <https://www.pfizer.com/science/clinical-trials/expanded-access>
- **Notes:** Pfizer Medical Information handles unsolicited product-information queries at 1-800-438-1985 Monday-Friday 9am-5pm ET. Expanded-access for investigational pan-KRAS agents is not posted as a standing program; case-by-case review applies.

Payer / coverage: Investigational. Sponsor-funded drug and protocol-mandated assessments under the trial; standard-of-care imaging and procedures bill insurance as usual.

Notes: Useful access positioning if a daraxonrasib slot is unavailable. No efficacy readout yet; the rank rides on mechanism class and the rare explicit G12R inclusion in eligibility.

6. anvumetostat (AMG 193) + daraxonrasib

Aliases: anvumetostat, AMG 193, AMG193, MTAPESTRY 103

Modality: small_molecule · **Regulatory:** Investigational. Both agents are pre-approval. Anvumetostat is in phase 1b; daraxonrasib carries FDA Breakthrough Therapy designation in PDAC but no approval. · **Guidelines:** Not listed in NCCN Pancreatic Adenocarcinoma v2.2026. The MTA-cooperative PRMT5 class has no NCCN evidence category in PDAC. · **Geographic scope:** United States and select EU sites; sponsor confirms by zip code. · **Verified:** 2026-05-12

Trial-only and gated on a still-pending MTAP test. MTAPESTRY-103 (NCT06360354) is recruiting and hits both the KRAS axis and the CDKN2A / MTAP axis in one regimen, but enrollment requires a homozygous MTAP deletion confirmed on NGS copy-number or IHC. The MTAP reflex on the existing tumor block is the gating workup.

Next steps

1. Order MTAP IHC (or NGS copy-number) on the archival tumor block. CDKN2A and MTAP co-delete in roughly 80-90% of homozygous CDKN2A losses because the two genes sit adjacent on 9p21, so this is a high-yield test.
2. If MTAP is co-deleted, email medinfo@amgen.com or call 866-572-6436 to find the closest MTAPESTRY-103 site with the daraxonrasib combination arm open.
3. If MTAP is intact, close this row and stay on the daraxonrasib monotherapy or chemo-combo lane.
4. Run MTAP testing in parallel with the NGS / ctDNA re-test for the daraxonrasib screening so the patient does not lose time waiting on either result.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06360354		recruiting	unconfirmed	Amgen Call Center; medinfo@amgen.com ; 866-572-6436

Manufacturer / sponsor contact

- **Company:** Amgen (anvumetostat) + Revolution Medicines (daraxonrasib)
- **Med info phone:** 1-800-772-6436
- **Med info email:** medinfo@amgen.com
- **Product info:** <https://www.amgenmedinfo.com/>
- **Compassionate / expanded access:** <https://www.amgen.com/about/compassionate-use>
- **Notes:** Amgen owns the IND on the combination via MTAPESTRY-103; medical-information and trial-screening inquiries route through medinfo@amgen.com or 1-800-772-6436. Daraxonrasib is supplied under collaboration; Revolution Medicines medical information at 1-844-2-REVMED can also field combination-specific questions.

Payer / coverage: Investigational. Sponsor covers both study drugs and protocol-mandated procedures. MTAP testing may be billable to insurance if ordered as part of routine NGS panel review.

Notes: The Bristol Myers Squibb successor program BMS-986504 / navlimetostat platform MountainTAP-5 (NCT07492680) opens later in 2026 and includes a daraxonrasib combination arm in PDAC; worth knowing about as a fallback if MTAPESTRY-103 fills before this patient screens.

7. avutometinib (VS-6766) + defactinib (FAK inhibitor) + gemcitabine/nab-paclitaxel

Aliases: avutometinib, VS-6766, defactinib, VS-6063, RAMP-205

Modality: small_molecule · **Regulatory:** Avutometinib + defactinib carries FDA accelerated approval (May 2025) for adult patients with recurrent KRAS-mutated low-grade serous ovarian cancer who have received prior systemic therapy, marketed as Avmapki Fakzynja Co-pack. No FDA or EMA approval in PDAC. Gem/nab is FDA-approved standard-of-care backbone. · **Guidelines:** Not listed in NCCN Pancreatic Adenocarcinoma v2.2026. The RAF/MEK clamp + FAK class is NCCN cat-2A for recurrent KRAS-mutant LGSOC after the 2025 accelerated approval, but the PDAC use is unlisted. · **Geographic scope:** United States, with select Asia-Pacific and EU sites historically. RAMP-205 specifically is active not recruiting. · **Verified:** 2026-05-12

Trial enrollment is the binding constraint. RAMP-205 is active not recruiting, so the dose-level-1 5/6 PR signal is informational rather than directly actionable. Avutometinib + defactinib is FDA-approved for recurrent KRAS-mutant LGSOC, which means off-label prescription is a theoretical fallback but is unlikely to clear payer review and is not supported by the manufacturer's expanded-access policy.

Next steps

1. Email clinicaltrials@verastem.com or call 1-877-VSTM-ONC (1-877-878-6662) to ask whether RAMP-205 has reopened or whether a successor PDAC study is planned.
2. If a successor trial opens, confirm whether adjuvant FOLFIRI counts as prior systemic therapy under the new protocol.
3. Off-label use of the LGSOC-approved Avmapki Fakzynja Co-pack for PDAC is technically possible but is not supported by Verastem's expanded-access policy and would face heavy payer review; the trial route is strongly preferred when available.
4. Treat this lane as informational context for what RAF/MEK clamp + FAK can do in KRAS-mutant disease rather than an actionable patient option this week.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05661282	1/2	active not recruiting	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Verastem Oncology
- **Med info phone:** 1-877-878-6662
- **Med info email:** medinfo@verastem.com
- **Product info:** <https://www.verastem.com/>
- **Compassionate use email:** clinicaltrials@verastem.com
- **Notes:** Verastem states publicly that it does not operate an expanded-access program outside its clinical trials; participation in an active trial is the only investigational-route. Adverse-event reports route through drugsafety@verastem.com. Trial-enrollment questions route through clinicaltrials@verastem.com.

Payer / coverage: The LGSOC indication is commercially available under the Avmapki Fakzynja Co-pack approval; payer coverage for an off-label PDAC use would require a biomarker-justified appeal and prior authorization, and most plans will not cover it without trial billing. Gem/nab is covered under its standard PDAC indication. Verastem patient-support resources for the LGSOC indication may not extend to off-label PDAC use.

Notes: RAMP-205 dose-level-1 abstract (5/6 PR at n=6) is unpublished as a full peer-reviewed paper. The exact binomial 95% CI on 5/6 runs roughly 36-100%, so a single non-responder collapses the point estimate. The cross-tumor LGSOC validation (Banerjee JCO 2025, ORR 44% in KRAS-mutant LGSOC) is the more durable evidence point.

8. daraxonrasib (RMC-6236)

Aliases: daraxonrasib, RMC-6236, RMC6236

Modality: small_molecule · **Regulatory:** Investigational. FDA Breakthrough Therapy designation for previously treated metastatic KRAS G12X PDAC (2024). No FDA, EMA, or PMDA approval yet; RASolute-302 phase 3 (NCT06625320) is the registrational trial and is active not recruiting. · **Guidelines:** Not listed in NCCN Pancreatic Adenocarcinoma v2.2026. Trial-enrollment principle for actionable alterations is the guideline-fit framing. ·

Geographic scope: United States plus select EU and Australia sites. Sponsor based in California; the call-center will identify the nearest open site by zip code. · **Verified:** 2026-05-12

Trial-only. The daraxonrasib slot the dossier ranks at #1 is on NCT05379985 (phase 1/2, recruiting); RASolute-302 phase 3 is biomarker-matched but no longer enrolling, and the RMC-GI-102 platform (NCT06445062) is the chemo-combination lane. All three studies funnel through Revolution Medicines medical information at medinfo@revmed.com / 1-844-2-REVMED.

Next steps

1. Email medinfo@revmed.com or call 1-844-2-REVMED to ask which RMC-6236-001 (NCT05379985) or RMC-GI-102 (NCT06445062) site is closest and currently accepting screening slots.
2. Send a plasma sample for central KRAS confirmation in parallel with the screening visit. The sponsor re-tests on its own platform regardless of the local NGS report.
3. Order an MTAP IHC or NGS copy-number reflex on the existing tumor block before the screening visit so a

positive result can flip the patient toward the MTAPESTRY-103 combination arm (NCT06360354) without losing time.

4. If a site near the patient does not open, ask the call-center whether an emerging satellite or referring-investigator arrangement is feasible before falling back to PF-07934040 or the chemo-combo lane.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05371285	1	recruiting	likely	Revolution Medicines, Inc.; medinfo@revmed.com; 1-844-273-8633
NCT06625320	2	active not recruiting	no	—
NCT06445062	1	recruiting	likely	Revolution Medicines; medinfo@revmed.com; 1-844-273-8633

Manufacturer / sponsor contact

- **Company:** Revolution Medicines
- **Med info phone:** 1-844-273-8633
- **Med info email:** medinfo@revmed.com
- **Product info:** <https://www.revolutionmedicines.com/>
- **Notes:** Revolution Medicines routes both medical-information and trial-screening inquiries through medinfo@revmed.com / 1-844-2-REVMED. No posted expanded-access program; pre-approval access is currently trial-only.

Payer / coverage: Investigational. Drug, study procedures, and central biomarker re-testing are sponsor-covered under the trial protocol. Standard-of-care procedures continue to bill the patient's insurance during participation.

Notes: Daraxonrasib carries FDA Breakthrough Therapy designation for previously treated KRAS G12X PDAC and the Wolpin NEJM 2026 cohort is the load-bearing efficacy datum on this axis. The patient is enrollable on biomarker and line; the gating axis is geography of an open site.

9. daraxonrasib (RMC-6236) + mFOLFIRINOX or gemcitabine/nab-paclitaxel

Aliases: daraxonrasib, RMC-6236, RMC-GI-102, RASmap

Modality: small_molecule · **Regulatory:** Investigational combination. Daraxonrasib carries FDA Breakthrough Therapy designation in PDAC but is not approved. The chemotherapy backbones (mFOLFIRINOX, gem/nab) are FDA-approved standard-of-care components used per protocol. · **Guidelines:** The combination is not listed in NCCN Pancreatic Adenocarcinoma v2.2026. The chemo backbones are NCCN cat-1 components; the pan-RAS layering is the unlisted half. · **Geographic scope:** United States plus select EU sites. Operationally narrower than the monotherapy lane because subprotocol openness varies site to site. · **Verified:** 2026-05-12

Trial-only. RMC-GI-102 (NCT06445062) is recruiting and layers daraxonrasib on mFOLFIRINOX or gem/nab in RAS-mutant PDAC. The case for this lane over monotherapy rests on the patient's oxaliplatin-naïve status and the

stated treat-to-remission goal; the RAMP-205 dose-level-1 abstract (5/6 PR at n=6) is the closest precedent but the dedicated combination-safety publication has not landed.

Next steps

1. Call Revolution Medicines medical information (1-844-2-REVMED) or email medinfo@revmed.com asking specifically about the RMC-GI-102 PDAC chemo-combination subprotocol and the nearest open site.
2. Clarify whether adjuvant FOLFIRI counts as prior systemic therapy. The answer routes the patient into the 1L or 2L+ subprotocol and affects backbone choice.
3. If residual neuropathy from prior FOLFIRI is borderline, ask the site to consider the gem/nab backbone rather than mFOLFIRINOX.
4. Run the same MTAP reflex test in parallel; if MTAP is co-deleted, the MTAPESTRY-103 combination arm becomes a competing trial-only lane that hits two features at once.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06447262		recruiting	likely	Revolution Medicines; medinfo@revmed.com ; 1-844-273-8633

Manufacturer / sponsor contact

- **Company:** Revolution Medicines
- **Med info phone:** 1-844-273-8633
- **Med info email:** medinfo@revmed.com
- **Product info:** <https://www.revolutionmedicines.com/>
- **Notes:** Same medical-information line as the monotherapy lane. The RMC-GI-102 platform is separately listed from the RMC-6236-001 phase 1/2; ask for the PDAC chemo subprotocol by name when calling.

Payer / coverage: Investigational. Daraxonrasib is sponsor-supplied. The chemotherapy backbone is typically billed to insurance under the FDA-approved indication (FOLFIRINOX or gem/nab in metastatic PDAC); confirm trial-billing arrangements with the site CTO so backbone components are not double-billed.

Notes: Higher response-rate ceiling than monotherapy in theory, but the load-bearing combination-safety publication is missing. Status in the dossier is considered_with_caveats with two persona dissents on missing safety data.

10. zoldonrasib (RMC-7977) + ivonescimab

Aliases: zoldonrasib, RMC-7977, ivonescimab, AK112

Modality: small_molecule · **Regulatory:** Investigational. Zoldonrasib is pre-approval; ivonescimab (Akeso AK112) is approved in China for first-line PD-L1-positive NSCLC and EGFR-mutant NSCLC post-TKI but has no FDA or EMA approval. The combination has no regulatory filing. · **Guidelines:** Not listed in NCCN, ESMO, or ASCO PDAC guidelines. Neither agent has a PDAC indication. · **Geographic scope:** United States plus select EU and Australia sites for the RAS(ON) backbone; ivonescimab access is broader in mainland China than in the

Trial-only and contingent on PDAC cohort openness. NCT07397338 is recruiting but the primary listed indications are NSCLC and CRC; whether a PDAC dose-expansion arm is open at a given site is the gating question. The case for keeping this lane on the page is that the patient's MSS plus TMB 4.1 closes every standard ICI door, and this is the only route to an immune-axis combination the patient could touch.

Next steps

1. Email medinfo@revmed.com asking whether NCT07397338 has an open PDAC dose-expansion cohort and at which sites.
2. If PDAC is not open, ask whether the sponsor is planning a PDAC-specific extension and on what timeline; the patient may be able to wait if disease tempo allows.
3. Before screening labs, confirm the rash-versus-immune-rash AE attribution plan with the site; pan-RAS class rash and PD-1xVEGF immune-rash overlap diagnostically.
4. Treat this lane as a fallback to the monotherapy and chemo-combo lanes given zero published clinical PDAC data on the combination.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07397338		recruiting	unconfirmed	Revolution Medicines Study Director; medinfo@revmed.com ; 1-844-273-8633

Manufacturer / sponsor contact

- **Company:** Revolution Medicines (zoldonrasib) + Akeso / Summit Therapeutics (ivonescimab US partner)
- **Med info phone:** 1-844-273-8633
- **Med info email:** medinfo@revmed.com
- **Product info:** <https://www.revolutionmedicines.com/>
- **Notes:** Revolution Medicines is the IND holder for the combination; route inquiries through medinfo@revmed.com. Summit Therapeutics holds North American development rights for ivonescimab via the Akeso partnership; medical-information questions specific to ivonescimab can route through Summit's investor / medical-affairs channels.

Payer / coverage: Investigational. Sponsor-supplied drugs; standard imaging and procedures bill insurance as usual under trial-billing arrangements.

Notes: Highest-ceiling preclinical option the patient could touch (KPC GEMM complete responses, Wasko Nature 2024), but zero published clinical PDAC efficacy data and three persona dissents on evidence, toxicity, and guideline-fit. The G12R microenvironment biology (Singhi 2025: lower-ERK and immune-enriched relative to G12D) is the most plausible reason this lane might behave differently than the published G12D KPC data.